



February 23, 2026

The Honorable Robert F. Kennedy Jr.
Secretary of Health and Human Services
U.S. Department of Health and Human Services
200 Independence Avenue, SW
Washington, DC 20201

Submitted electronically via www.regulations.gov

RE: HHS Health Sector AI RFI – Accelerating Use of AI (RIN: 0955-AA13)

Dear Secretary Kennedy:

On behalf of Consensus Cloud Solutions, we are writing in response to the Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care.

Introduction

Consensus Cloud Solutions is the largest digital cloud fax platform in the country and over the last 25 years, we have expanded to multiple interoperability protocols and have experienced firsthand the success and opportunities in the usage of intelligent AI supported cloud fax.

High-quality patient care is severely constrained when essential clinical information remains inaccessible as the information is trapped in unstructured formats such as faxes, PDFs, and scanned images. Although the health care industry continues to pursue full digital maturity, an estimated 70% of all healthcare data remains unstructured, with the burden disproportionately concentrated in rural and underserved markets. As a result, providers must manually review and re-enter data to inform care, leading to delays, errors, and significant administrative burdens.

The unstructured data problem is not going away. In many rural areas and underserved markets, fax machines are still the primary mechanism for care coordination. While cloud-based fax solutions represent progress, these providers still transmit unstructured data

Therefore, critical care content is not easily accessible as the data is buried in the pile of faxes, PDFs, and scanned images that need to be accessed before patient treatment is delivered.

Recommendations

Consensus recommends a federal strategy that recognizes Intelligent AI extraction as a vital, low-risk category of AI that bridges the "digital divide" by transforming legacy document streams into structured, actionable clinical intelligence.

1. The Necessity of Logic-Based AI Extraction

Many workflow solutions rely solely on traditional Optical Character Recognition (OCR), which frequently fails due to its inability to interpret clinical context. Consensus' Clarity AI solution utilizes Natural Language Processing (NLP), Machine Learning (ML), and advanced AI techniques to deliver contextual interpretation rather than simple character reading.

- **Error Correction through Cross-Reference:** For example, Clarity AI detects logic inconsistencies, such as recognizing that a patient's birthdate of 1918 extracted from a pediatric record is a logical error. By cross-referencing the Electronic Health Record (her) the correct patient's date of birth can be added. Similar corrections apply to mistyped NPI numbers by formatting anomalies such as invalid phone numbers. These capabilities significantly reduce provider burden and downstream errors.
- **Traceable Source of Truth:** To foster provider trust, every piece of data extracted via **Clarity** AI includes a "click-back view" to the original source document. This ensures that the AI serves as an assistant to, rather than a replacement for, clinical judgment.

2. Workflow Impact: Ending the "Referral Black Box"

Administrative burden remains a primary driver of clinician and administrative staff burnout. Manual referral processing takes an average of **70 minutes per referral**¹, leading to 40-65% of referrals being abandoned.² Resulting delays contribute to patient no-shows and “referral leakage” where patients are lost to follow-up or seek care elsewhere.

- **The Clarity Solution:** Our AI-driven extraction fixes errors and can now map the structured data with accurate clinical content directly into the receiving end intake workflows and scheduling. By reducing processing time from days to minutes, we eliminate "referral leakage" and can move patients from referral to scheduling within 24 hours. This accelerates access to care, improves patient trust, and significantly reduces administrative friction for providers.

3. Solving the Prior Authorization "Medical Injustice"

According to a recent AMA survey³, **94% of physicians** report that prior authorization (PA) causes significant delays in care, with **78% of patients** eventually abandoning treatment due to these hurdles. On average, a medical practice spends **12 hours per week** just on PA paperwork, which is a manual "black hole" that fuels clinician burnout and patient risk. While many PA solutions focus on payer-side decision workflows, the primary bottleneck with payers often begins at the provider-level, specifically in the documentation phase. Providers struggle to find specific clinical evidence and correct ICD-10 codes that are hidden in hundreds of pages of faxes and PDFs and are required to satisfy a payer's medical necessity rules. At Consensus Cloud Solutions, our technology can assist in reducing the prior authorization hurdles and burdens that clinicians and patients face.

¹ Bevey Miner, Bolstering Referral Management with a Streamlined Approach Using Digital, AI-Powered Fax, Medical Economics (Nov. 24, 2025), <https://www.medicaleconomics.com/view/bolstering-referral-management-with-a-streamlined-approach-using-digital-ai-powered-fax>.

² Referral Tracking Gap: Why You Can't Track Referrals, SocialRoots.ai (Dec. 15, 2025), <https://www.socialroots.ai/blog/closed-loop/referral-tracking-gaps>.

³ Tanya Albert Henry, Exhausted by prior auth, many patients abandon care: AMA survey, AMA News Wire (July 18, 2024), <https://www.ama-assn.org/practice-management/prior-authorization/exhausted-prior-auth-many-patients-abandon-care-ama-survey> (last visited Feb. 23, 2026).

- **Intelligent Documentation Assembly: Clarity** uses AI extraction on unstructured PAs to correct errors and automatically extract fields necessary for the PA workflow.
- **Reduced Decision Time:** By presenting "clean," structured data to payers in a digital format (e.g., x12, FHIR), Consensus-powered workflows can reduce PA processing time from **days to minutes**.
- **Eliminating "Blind" Denials:** Our technology ensures that the *first* submission is clinically complete. This prevents the cycle of "incomplete documentation" denials that force patients to wait weeks for potentially life-saving care and provider frustration working through denials.

4. Response to Specific RFI Questions

Question 2: Regulatory & Payment Policy

As HHS continues to incentivize effective AI use, HHS should consider payment for data extraction and reconciliation as part of the proactive “Incentivized Integration” model and not only focus on a reactive information blocking posture.

Current CMS policies do not recognize or reimburse the substantial administrative effort required to convert external unstructured data into usable data. Consensus recommends establishing **Value-Based Care (VBC)** bonuses for the use of AI-driven tools that automate this process, reduce error rates, and improve care coordination.

Question 8: Enhancing Interoperability Benchmarks

Interoperability works only when data can be sent, received, found, and integrated into downstream workflows. Today, the “found” and “integrated” functions are often missing, limiting the utility of national exchange networks and impeding progress toward ensuring all Americans have access to their data. The success of frameworks such as TEFCA and QHINs depends on the quality and structure of the underlying data. Without unlocking the large volume of unstructured information held by rural and small providers, population health will continue to reflect data gaps and systemic biases.

We urge HHS to set standards for AI-automated data mapping along with the FHIR initiative. Query is only as successful as the quality of the data shared; extracting and structuring data from unstructured documents needs to be a part of the federal initiative. This way rural and underserved providers can share data and help us achieve the goal of all Americans having access to their data.

Question 10: Research & Development

We applaud HHS in seeking feedback on creating new, long-term market opportunities and integrating AI into care delivery. Many advanced AI solutions are out of reach for providers in rural health and underserved markets. These providers often lack the capital, IT infrastructure, and the technical staff to implement AI within their practice. While we appreciate the Rural Health Transformation Program goal that aims to resolve this issue, significant gaps may remain.

HHS should prioritize R&D that targets the "excluded" providers, such as the rural clinics, substance abuse centers, and birthing centers, who were carved out of early meaningful use initiatives in the years past.

- **The Digital On-Ramp:** R&D should focus on "Digital Maturity Acceleration" via public-private partnerships. Consensus proposes a **"Fax-to-FHIR" Highway**, where federal funding supports the deployment of cloud-based AI extraction tools for rural and underfunded small practices along with integration into other systems and workflows. This solution allows providers to use a low-cost secure platform, and the technology does the extraction and conversion.
- **Technical Assistance:** Funding should be allocated not just for the platform, but for the technical assistance required to integrate AI-extracted data into more advanced end points, ensuring rural networks have parity with urban health systems.

Conclusion

We look forward to continued collaboration in the development of the Department's AI strategy. If you have any questions related to our letter, please contact Bevey Miner, Healthcare Strategy & Policy at bevey.miner@consensus.com.

Sincerely,

A handwritten signature in black ink, reading "Bevey Miner", is displayed on a light blue rectangular background.

Bevey Miner
Executive Vice President, Healthcare Strategy & Policy
Consensus Cloud Solutions